

SENATE BILL 2036

By Harshbarger

AN ACT to amend Tennessee Code Annotated, Title 8 and Title 56, relative to health insurance.

BE IT ENACTED BY THE GENERAL ASSEMBLY OF THE STATE OF TENNESSEE:

SECTION 1. This act is known and may be cited as the "Stabilizing Healthcare Access with Reimbursement Protections (SHARP) Act."

SECTION 2. Tennessee Code Annotated, Title 56, Chapter 7, Part 10, is amended by adding the following as a new section:

(a) As used in this section:

(1) "Annual benefit maximum" means the dollar cap on total covered services paid per insured individual, per policy year, where applicable;

(2) "Health insurance entity":

(A) Means an insurance company, managed care organization, health maintenance organization (HMO), or third-party administrator offering or administering health coverage and subject to regulation by the commissioner; and

(B) Does not include a plan governed exclusively by the federal Employee Retirement Income Security Act of 1974 (ERISA)(29 U.S.C. § 1001, et seq.);

(3) "Inflation index" means the annual percentage increase in the consumer price index for all urban consumers (CPI-U), as published by the United States bureau of labor statistics; and

(4) "Reimbursement rate" means the maximum allowable amount payable by a health insurance entity for covered healthcare services rendered by in-network providers.

(b) Beginning January 1, 2027, and January 1 of each year thereafter, each health insurance entity shall:

(1) Adjust all in-network reimbursement rates upward by no less than the annual percentage increase in the inflation index for the preceding calendar year, which may be applied by provider type or specialty or service category;

(2) Adjust annual benefit maximums under individual and group policies upward by no less than the annual percentage increase in the inflation index for the preceding calendar year, which may be applied by provider type or specialty or service category; and

(3) Cap each annual increase at four percent (4%), even if the inflation index exceeds four percent (4%), unless otherwise approved by the commissioner of commerce and insurance.

(c) This section does not apply to:

(1) Policies covering fewer than one hundred (100) enrollees;

(2) Self-insured plans governed exclusively by the federal Employee Retirement Income Security Act of 1974 (ERISA)(29 U.S.C. § 1001, et seq.);

(3) Medicare advantage or TennCare managed care plans operating under federal waiver authority;

(4) Out-of-network provider reimbursement rates; or

(5) A plan granted a temporary exemption by the commissioner under subsection (f).

(d)

(1) This section applies prospectively to all applicable policies issued, amended, or renewed on or after January 1, 2027.

(2) Adjustments must be included in provider contracts, reimbursement schedules, or benefit documents provided to the commissioner as part of rate or form filings.

(e)

(1) The commissioner shall develop a public annual report identifying health insurance entities in compliance with this section.

(2) Health insurance entities shall file with the commissioner, no later than April 1 of each year, documentation of compliance with the inflation adjustment requirement.

(3) The commissioner shall monitor and review rate filings submitted by health insurance entities to ensure that adjustments in premiums are proportionate and justified in relation to the inflation index-based reimbursement updates.

(f)

(1) A health insurance entity may request a waiver from the requirements of this section by submitting to the commissioner an actuarial certification demonstrating that compliance would cause material financial hardship or jeopardize solvency.

(2) The commissioner may grant a full or partial exemption for no more than one (1) calendar year per filing.

(g) A health insurance entity may voluntarily implement a one-time catch-up adjustment to any in-network reimbursement rates or annual benefit maximums that have remained unchanged for at least five (5) consecutive calendar years prior to the

effective date of this section. The catch-up adjustment may equal the cumulative CPI-U change over that five-year period, subject to a cap of twenty percent (20%). Voluntary adjustments must be filed with the commissioner within sixty (60) days of implementation. A health insurance entity may prioritize adjustments for rural and small practices.

(h)

(1) A health insurance entity may voluntarily implement a qualifying catch-up adjustment under subsection (g) on or before December 31, 2029.

(2) Beginning January 1, 2030, a health insurance entity shall implement a catch-up adjustment to any in-network reimbursement rates or annual benefit maximums that:

(A) Have remained unchanged for five (5) consecutive calendar years prior to the effective date of this section; and

(B) Fall within a line of coverage subject to regulation under this chapter.

(3) The mandatory catch-up adjustment under subdivision (h)(2) must equal the cumulative CPI-U increase for the frozen period, subject to a cap of twenty percent (20%).

(4) The commissioner shall establish procedures to monitor compliance, review filings, and enforce catch-up adjustments pursuant to this subsection (h). A health insurance entity that maintains a full provider network and makes good-faith efforts to comply with this section is eligible for safe harbor protections, subject to review by the attorney general and reporter or designated legal counsel.

(i)

(1) The commissioner may prioritize review and enforcement of this section in counties identified as provider shortage areas or health professional shortage areas.

(2) The commissioner shall ensure that small or rural providers receive equitable consideration in inflationary adjustments.

(j) A health insurance entity shall not reduce the size of in-network provider panels in a discriminatory manner as a response to inflationary adjustments. The commissioner shall review provider network adequacy as part of rate oversight under this section.

(k)

(1) The department of commerce and insurance is authorized to allocate appropriations, hire additional staff, and procure technology necessary to support CPI tracking, compliance review, and public transparency.

(2) The commissioner shall issue a transition timeline and guidance no later than ninety (90) days after the effective date of this act, including grace periods for systems integration.

(l) The commissioner shall encourage insurers to conduct and submit multi-year rate reviews in conjunction with inflation index-based adjustments.

(m) The commissioner of commerce and insurance is authorized to promulgate rules to implement this section. The rules must be promulgated in accordance with the Uniform Administrative Procedures Act, compiled in title 4, chapter 5.

SECTION 3. This act takes effect upon becoming a law, the public welfare requiring it.